

Vision Loss or Change - Male (Adult)

Algorithm 1313 | Adult (18 years and older) | Gender at birth: Male

Definition

- UAT-only adult and pediatric assessment of new vision loss or change, with sudden loss, severe painful eye disease, and associated neurological emergencies routed directly to emergency care.
- Adult patient.
- Male.

Initial assessment

1. [OPEN_TEXT] What changed: complete or partial loss, blur, double vision, flashing lights, new floaters, dark curtain or shadow, missing field, color change, or inability to fix and follow?
2. [DURATION] What exact time did it start, was onset sudden or gradual, and is it constant, worsening, or now resolved?
3. [YES_NO] When each eye is covered separately, does the change affect one eye, both eyes, or the same side of the visual field?
4. [YES_NO] Is there severe eye pain, a red eye, headache, nausea or vomiting, unequal pupils, eye injury, chemical exposure, recent eye surgery, or contact-lens use?
5. [YES_NO] Is there facial droop, arm or leg weakness or numbness, speech difficulty, severe imbalance, confusion, seizure, or severe sudden headache?
6. [YES_NO] For a baby or child, is there a sudden failure to fix or follow, new abnormal eye movement or squint, white or absent red reflex, or caregiver concern that the child cannot see normally?

Triage and redirect questions

1. Disposition level 100

Did the eye symptoms begin after a chemical splash or chemical exposure?

The Eye - Chemical In family is source-only and has no generated target. Chemical-associated vision symptoms remain a direct emergency disposition with immediate irrigation for a splash without suspected penetration, Qatar 999, and no self-driving.

1. Disposition level 100

Is there sudden complete or partial vision loss in one or both eyes or a visual field; sudden severe eye pain, especially with red eye, headache, nausea or vomiting; or vision change with stroke signs, severe headache, seizure, eye trauma, or chemical exposure?

NHS.UK treats sudden vision loss and sudden severe eye pain as emergencies. Associated focal neurological symptoms may indicate stroke, while a painful red eye with systemic symptoms may be another sight-threatening emergency. Children may not describe loss reliably, so a sudden objective change in visual behavior must fail closed.

1. Disposition level 70

With no emergency feature above, is there new or worsening blur or double vision, first or suddenly increased flashes or floaters, a dark curtain or shadow, eye pain, red eye, light sensitivity, abnormal eye movement, or a persistent visual concern in a child?

NHS.UK advises urgent assessment for new flashes, floaters, dark curtain or shadow, blur, pain, or other new changes because retinal and other ocular disease can permanently affect vision. Adults and children require an age-appropriate in-person examination.

Advice

Qatar emergency vision response

- Call Qatar emergency services on 999 and record the exact last-known-normal time. Do not allow the person to drive, eat or drink while emergency assessment is being arranged, or put unprescribed drops or medication in the eye. Keep them safely supervised. For a chemical splash without suspected penetration, protect the rescuer, remove contact lenses only if easy, and immediately rinse the open eye with a gentle flow of clean, room-temperature water for at least 20 minutes; continue irrigation while help is arranged and take the product container without delaying care. Do not try to neutralize the chemical. If penetration, an embedded object, a cut, fluid leak, or open-globe injury is possible, do not irrigate, rub, press, patch, remove the object, or put drops or ointment in the eye; loosely protect it without pressure. Do not delay emergency transport. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: vision does not return; pain worsens; stroke sign; vomiting or reduced consciousness; new seizure.

Urgent age-appropriate ophthalmic assessment

- Arrange same-day in-person assessment through an approved Qatar adult or pediatric ophthalmology pathway. Do not drive while vision is affected. Avoid contact lenses and unprescribed eye drops. Call 999 immediately for sudden loss, severe pain, stroke signs, severe headache, vomiting, confusion, seizure, or rapid worsening.
- Call Back If: sudden or complete vision loss; severe eye pain; stroke signs; rapid worsening; child stops fixing, following, or using an eye.

First aid

- Call Qatar emergency services on 999 and record the exact last-known-normal time. Do not allow the person to drive, eat or drink while emergency assessment is being arranged, or put unprescribed drops or medication in the eye. Keep them safely supervised. For a chemical splash without suspected penetration, protect the rescuer, remove contact lenses only if easy, and immediately rinse the open eye with a gentle flow of clean, room-temperature water for at least 20 minutes; continue irrigation while help is arranged and take the product container without delaying care. Do not try to neutralize the chemical. If penetration, an embedded object, a cut, fluid leak, or open-globe injury is possible, do not irrigate, rub, press, patch, remove the object, or put drops or ointment in the eye; loosely protect it without pressure. Do not delay emergency transport. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay

emergency care.

References

- Qatar Ministry of Public Health, "Healthcare Services in Qatar", <https://sportandhealth.moph.gov.qa/EN/faninfo/Pages/HealthcareServicesInQatar.aspx> (accessed 2026-07-25)
- NHS.UK, "Vision loss", <https://www.nhs.uk/conditions/vision-loss/> (page last reviewed 28 August 2025)
- NHS.UK, "Floaters and flashes in the eyes", <https://www.nhs.uk/symptoms/floaters-and-flashes-in-the-eyes/> (accessed 2026-07-25)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: 8ef9a010bc5c25e2c6ab54183fcb51d3a4ebf6e43e234998c7eaffb2bbb2e2e4 | Status: BLOCKED_BY_RESEARCH_GAP